



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض رابعياً : إيمان محمد فكرى محمود
المحافظة : القاهرة
الحى / المدينة : الظاهر
رقم البطاقة : 26612310201948 رقم التليفون : 01223624982
العنوان : -
العمر (تاريخ الميلاد) : 31-12-1966 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 30-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....) المقرر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي العلاج بعقار TDM1 ويردي [14121] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض
بالمستشفى قبل الصرف.

أعضاء اللجنة

د.ا. / د.ا. / د.ا. / د.ا. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: إيمان محمد فكري محمود
National ID: 26612310201948



تقرير طبي

المريض

تحية طيبة وبعد،،

بشأن تقرير طبي للمريض

إهداء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان توضح لهما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة التناول :
--- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالبدن و تحتاج الي IV (trastuzumab emtansine) TDM1

الوقت :

التاريخ :

05-07-2026

الطبيب المعالج :
د. إيمان لاشين



MDT FORM 22/12/2025 G

Name	إيمان محمد محمد	MRN	٣٥٦/٢-٢٥
Age	58	ID	٢٦٦٢١-٢-١٩٤٨
Menopausal status		Phone	01223624982
NRS SCORE		Marital status	
BMI		Family history	- UELH
CO morbidity	m-hce	REASON OF MDT	Post + o.f

Clinical Data

BX: Initial doc CT

ER: PR: HER 2: Ki67:

Sonomammography:

S18 Q2 + Sm + slw D (Post + vnet) TNM

Mets Workup:

Deep margin free
Nipple & areola -> free
(0/4) no metastatic deposits

Recommendations:

Post op with fur. SDC
cellulitis 40%

Amr/Amr
2-20/12/11

MDT OUTCOME & PLAN

Adh Radiotherapy + post op
+ Araden + 2h of bus

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Amr/Amr		Nor R 22/12/2025			

Name: ایمان محمد فکری محمود		
MR. : 3122		
Detailed Medical Report		
		السید
تحية طيبة وبعد,,		
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض	
بقسم:	رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمناقشة وجد انه		
The patient is a 58-year-old individual with a history of right-sided breast cancer, clinical stage T3N0M0, invasive ductal carcinoma, grade 2, estrogen receptor 8/8 positive, progesterone receptor 4/8 positive, HER2 3+ positive. The patient underwent right-sided mastectomy on 5 November 2025 followed by radiation therapy (RT) with a surgical margin status of ypT1 ypN0/4 and an estimated ejection fraction of 66 % on 3 February 2025. Adjuvant therapy was initiated with KADCYLA 260 mg and ARIMIDEX. Subsequent treatment planning indicated continuation with KADCYLA and ARIMIDEX for cycle 4, followed by KADCYLA for cycle 5; with pharmacy coordination and a repeat echocardiogram scheduled. The pharmacy note on 9 April 2026 confirmed administration of cycle 5 KADCYLA 260 mg together with ARIMIDEX. On 30 April 2026 the patient attended a pharmacy visit for cycle 6 KADCYLA plus ARIMIDEX, and a new echocardiogram was ordered prior to the next cycle. On 23 May 2026 the patient's ejection fraction was documented at 62 % on 13 May 2026; the port was removed because KADCYLA was unavailable at that time. On 20 June 2026 the record indicated that KADCYLA remained unavailable, and the patient continued ARIMIDEX with supportive treatment. The pharmacy note on 28 June 2026 detailed preparation for cycle 7 KADCYLA 260 mg, addition of ANASTROZOLE, and a supportive echocardiogram, with the most recent ejection fraction still 62 %. On 9 August 2026 the patient was scheduled for cycle 9 KADCYLA; laboratory results were accepted, and ARIMIDEX with supportive therapy and a repeat echocardiogram were planned for the next visit. The pharmacy visit on 20 August 2026 recorded the patient's weight as 92 kg and confirmed administration of cycle 9 KADCYLA 260 mg with ANASTROZOLE; laboratory values from 9 August were accepted. On 9 September 2026 the ejection fraction was 66 %; dexamethasone was prescribed for osteoporosis prophylaxis, and the patient was prepared for cycle 10 KADCYLA 260 mg with ARIMIDEX and a proton pump inhibitor. The patient has now completed ten cycles of KADCYLA.		
تحريراً في ,		
التاريخ :	20-09-2026	الوقت :
الطبيب المعالج :	dina makram	
يعتمد ,		

Patient Name :	الأستاذة / ايمان محمد فكرى محمود محمد	Request No. :	13002349
Patient No. :	26612310201948	Request Date :	15/02/2025
Path No. :	SR-261/25	Verification Date :	04/03/2025
Sex :	Female	Financial Category :	جراحة روسي - لفقة الدولة
Age :	58 Y	Department :	Out Patient
Referring Doc :		Registered by :	marwa mohamed youss

Pathology Report

Clinical data:
Breast mass.

Nature of specimen:
Slide and block revision.

Gross examination:

- Three paraffin blocks labelled by (8367-24), (505-25) and (1647-24).
- Six immunostained slides for (ER, PR, HR2 and three slides for HER2 ISH).
- Four referred films, two of which labeled by (1647-24) and other two labeled (1635-24).

Microscopic examination:

Slides and block labeled by (505-25):

- * Histological type: Invasive duct carcinoma .
- * Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+1) Grade II .
- * DCIS: Minimal.
- * Angiolinvasion: Could not be detected within the limits of biopsy recieved
- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Weak nuclear reaction in <1% of tumor cells (score 2/8).
- Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).
- SISH :Amplified, Her2 copy number:5.2, Her2/CEP 17: 4.2.

Slides and block labeled by (8367)

- * Histological type: Invasive duct carcinoma .
- * Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+1) Grade II .
- * DCIS: Focal .
- * Angiolinvasion: Could not be detected within the limits of biopsy recieved.
- * Microcalcifications: Positive.

Verified by :
dr.esraa magdy

Printing Date : 27/08/2025 09:40:38

Page 1 of 2

Printing By :Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mir El Qadema - Cairo, Egypt

Patient Name : الأستاذ / أيمن محمد فكري محمود محمد	Request No. : 13002349
Patient No. : 26612310201948	Request Date : 15/02/2025
Path No. : SR-281/25	Verification Date : 04/03/2025
Sex : Female	Financial Category : جويشك زومبي - نفقة الدولة
Age : 58 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

- * IPT (Positive control showed appropriate reactivity);
 - ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
 - PR: Moderate nuclear reaction in 5% of tumor cells (score 4/8).
 - Her2: Moderate complete membrana reaction in >10% of tumor cells (score 2+).
 - SISH: Amplified.
- Films and blocks labeled by (1635, 1647-24): Hemorrhagic smears free of atypical cells.

Diagnosis:

Breast mass, slide and block revision:

Referred material labeled as (505-25)

Invasive duct carcinoma, Grade II.

- ER: Positive (score 8/8).
- PR: Negative (score 2/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified.

Referred material labeled as (8367)

- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified.

Referred films: Hemorrhagic smears free of atypical cells.

Dr. Rana Fathy.

Dr. Duaa Muhammed

Verified by :

dr.esraa magdy

Printing Date : 27/08/2025 09:40:38

Page 2 of 2

Printing By : Mrs. Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - نهر النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Mistr El Qadema - Cairo, Egypt

Patient Name	: الأستاذ / إيمان محمد فكرى محمود محمد	Request No.	: 13003821
Patient No.	: 26612310201948	Request Date	: 06/12/2025
Path No.	: SR-1765/25	Verification Date	: 09/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 58 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, Post NACT.

Nature of Specimen: Referred slides (Right simple mastectomy & Sentinel lymph nodes biopsy).

Gross Examination:

Six referred slides were received labelled by (204870-25) & sublabelled from (A:F).

Microscopic examination:

- Histological type: Residual viable invasive duct carcinoma (Cellularity 40%).
- Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- Ductal carcinoma in situ: Absent
- Angiolinvasion: Absent.
- Perineural invasion: Absent.
- Nipple and areola: Free.
- Deep margin: Free.
- Sentinel lymph nodes: Four lymph nodes with no metastatic deposits, (0/4).
- Histologic response to therapy: Pathologic partial response to therapy.

Diagnosis:

Right breast cancer (Post-NACT), Referred slides (Right simple mastectomy & Sentinel lymph nodes biopsy):

Residual viable invasive duct carcinoma, Grade 2 (Cellularity 40%).

- Ductal carcinoma in situ: Absent.
- Nipple and areola: Free.
- Deep margin: Free.
- Sentinel lymph nodes: Four lymph nodes with no metastatic deposits, (0/4).

** N.B; For proper staging & calculation of tumor cancer burden, please refer to the original report.

Verified by :

Dr.Duaa Mohamed



Patient Name	: الأستاذة / ايمان محمد فكرى محمود محمد	Request No.	: 13003821
Patient No.	: 26612310201948	Request Date	: 06/12/2025
Path No.	: SR-1765/25	Verification Date	: 09/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 58 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Signature

Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed



MRN: 10124255695
Patient Name: EMAN MOHAMMED FAKH MAHMOUD
D.O.B: 31/12/1966
IC No.: 26612310201948
Sex: Female
Location: External Clinic

بهيّة
في شهر كل ست مصرية

SURGICAL PATHOLOGY

Specimen examination - (1 Site)

Specimen Id 2511050090
Accession No Z-SUR25-204870
Sample Type Tissue

Collection Date 05/11/2025 12:08 PM
Received Date 05/11/2025 12:08 PM
Doctor MOHAMMED FAWZY AHMED
MOHAMMED SALEM

Clinical Information

Rt breast simple mastectomy and sentinel lymph node biopsy for a mass previously diagnosed as invasive duct carcinoma (path.no. SUR24-8367), post chemotherapy.

Gross Picture

Received two formalin-fixed specimen, labeled with the patient's name, MRN and "laterality RT".

(1)-Rt. simple mastectomy specimen (21×15 cm) with a two sutures "long=lateral" & "short=superior", covered by skin ellipse (19×12 cm), that included unremarkable nipple. Serial sections showed a rubbery white area (8×7 cm) including firm white mass (1.5×1 cm), located at (U.I.Q), 1 cm from the skin and 2.7 cm from the deep margin. The rest of the breast tissue is thick and white.

(2)-RT. SLN: four lymph nodes, the largest one was 1×0.7 cm. (N.S)

Microscopic Picture

Sections examined revealed wide areas of fibrohyalinosis around foci of viable invasive duct carcinoma, grade 2 with overall tumour viability of 20%. No evidence of in-situ component. No lymphovascular emboli detected. Nipple is free of tumour cells and Paget's disease. Least deep margin of excision is negative for tumour spread. All four dissected lymph nodes are negative for metastatic spread (0/4).

Assessment of tumour response to neoadjuvant chemotherapy : Tumour viability :

20%, Residual Cancer Burden class : II, Residual Cancer Burden Index : 2.229

Diagnostic Summary

Tumour type:

Invasive duct carcinoma

Specimen type:

Simple mastectomy and sentinel lymph node biopsy

Laterality

Right

Location

Upper Inner quadrant

Lesion Size

Invasive tumour size/s (mm):

T1c : [10 - 20] mm

Histological grade

Mitosis score (1-3):

2

Tubular formation score (1-3):

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في شهر كل ست مصرية

3
Nuclear grade (1-3):

2
Histological grade (1-3):

2
Peritumoural lymphovascular invasion

Absent
Skin ulceration

absent
Involvement of muscle

Absent
DCIS in specimen

Absent
Therapy Response:
Residual Cancer Burden

2
Diagnosis

Rt breast. Simple mastectomy and sentinel lymph node biopsy:

- Status post chemotherapy for invasive duct carcinoma; grade 2 [ypT1cN0Mx].
- **Assessment of tumour response to neoadjuvant chemotherapy : Tumour viability : 20%, Residual Cancer Burden class : II, Residual Cancer Burden Index : 2.229**
- Resection margin : negative
- Sentinel lymph node biopsy : negative for metastasis (0/4).

Category

Malignant primary site
Signature

Dr. Hend Elghareeb
Co-signed by:

Dr. Al Shimaa Zakaria, MD

Reported and electronically endorsed by
Mona Mohamed Ali Elabraq

مراكز ومستشفيات بهية

Validated on 11/11/2025 02:52 PM

Print Date: 12/11/2025 02:51 PM

2
2 شارع الخليفة مفرع من شارع الهرم - الجيزة
- الحى الاول - شارع ٤ - بجوار قسم الشيخ زايد اول - مدينة ٦ أكتوبر
hospitals@baheya.org

Page Number
www.baheya.org
[DOCTYPE:PATHOLOGY_REPORT]

Patient Name: Eman Mohamed Fekry Mahmoud Mohamed	Date : 13/07/2025
Exam : MAMMOGRAPHY BOTH	Age : 58 Year(s)
Medical ID: 326/2025	Pat. ID: 99378

Bilateral Digital soft tissue mammography with high-resolution breast sonography revealed:

Clinical summary: 58 years old female, underwent neoadjuvant chemotherapy and targeted therapy for pathologically proved right IDC. Post-NACT re-evaluation study.

Findings:

Right breast:

- Heterogeneously dense breast parenchyma that may obscure small lesions (ACR c).
- Upper central middle third architectural distortion with overlying segmental suspicious microcalcifications overlying and area measuring 32 x 11 mm seen till mm from nipple with no definite speculations or masses.
- Ultrasound demonstrated heterogeneous breast with mottled appearance.
- Upper central, middle third area of parenchymal distortion with no definite masses. Anterior to such area few prominent ducts are noted not exceeding 3 mm in caliber with microcalcifications seen within, yet with no abnormal vascularity on color Doppler study; such changes were detected till 6 mm from nipple.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast:

- Heterogeneously dense breast parenchyma that may obscure small lesions (ACR c).
- No speculations, masses asymmetry, distortion or suspicious microcalcifications.
- Few scattered rounded benign macrocalcifications.
- Ultrasound demonstrated heterogeneous breast parenchyma with mottled appearance with no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 6): Complete radiological response of 12 o'clock malignant mass lesion with still noted suspicious microcalcifications as described.

Left breast (BIRADS 2): breast adenosis with scattered benign calcifications.

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	
BIRADS 6: Pathologically proven malignant	98-100%

ECHO REPORT

NAME:

عبد الرحمن بن عبد الله

DATE: 7/9/2026

ID NUMBER:

/

SEX: ♀

AGE:

EF	66%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.4	3.5-5.8cm	AO.R	2.5	3-3.7cm
LVEDD		2-4cm	LA	3.8	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	1.0	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions

normal

LV Wall Thickness

normal

LV Diastolic Function

DD B Grade I

SWMA

normal

LA and Ao Root Dimensions

normal

Mitral Valve

normal

Aortic Valve

normal

Tricuspid Valve

normal

Pulmonary Valve

normal

Right Side and Systolic Function

normal

Intracardiac Masses and Effusion

normal

DR.

جستافى

Name: EMAN MOHAMED FEKRI
Patient ID: 0508202609
DOB: 31 December 1966

Sex: Female
Ethnicity: Hispanic

Height: 169.0 cm
Weight: 92.0 kg
Age: 59

Referring Physician:

Scan Date: 05 August 2026 - A0805260T

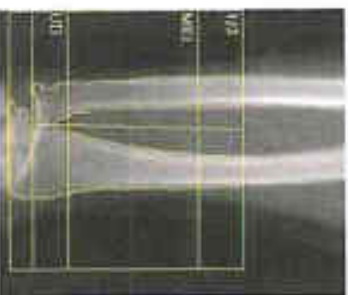
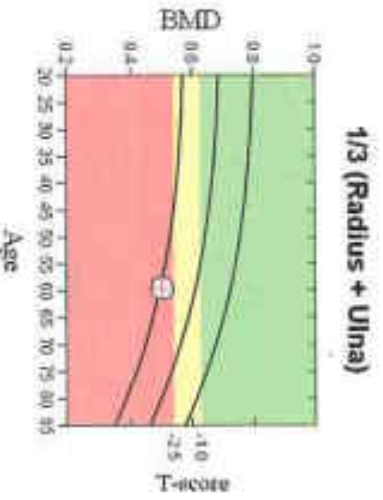


Image not for diagnostic use
228 x 103
DAP: 1.6 cGy*cm²



Radius + Ulna	BMD (g/cm³)	T-score	Z-score
1/3	0.504	-3.1	-1.8

WHO Classification: Osteoporosis
Fracture Risk: High

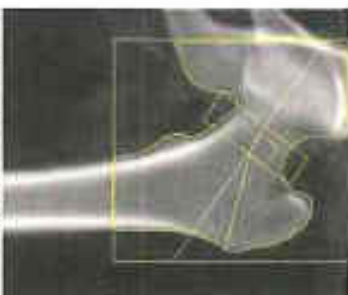
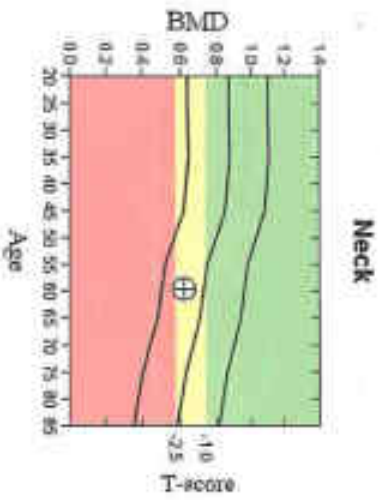


Image not for diagnostic use
97 x 103
NECK: 49 x 15
DAP: 1.7 cGy*cm²



Region	BMD (g/cm³)	T-score	Z-score
Neck	0.620	-2.1	-0.9
Total	0.726	-1.8	-0.8

WHO Classification: Osteopenia

10-year Fracture Risk*	
Major Osteoporotic Fracture	8.0%
Hip Fracture	0.6%
Reported Risk Factors:	
US (if female): Neck BMD -0.026, BMD -0.22	

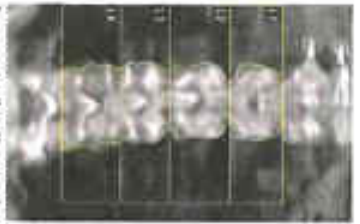
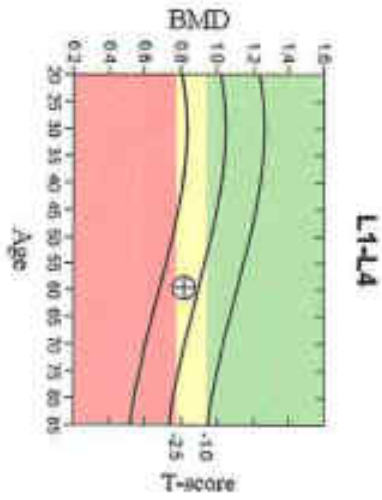


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116 x 129
DAP: 2.2 cGy*cm²



Region	BMD (g/cm³)	T-score	Z-score
L1-L4	0.816	-2.1	-0.7
L2-L4	0.824	-2.3	-0.9

WHO Classification: Osteopenia
Fracture Risk: Increased

Scan Date: 05 August 2026 - A0805260R

HOLOGIC®

ECHO REPORT

NAME:

عبد الرحمن بن عبد الله

DATE: 7/9/2026

ID NUMBER:

/

SEX: ♀

AGE:

EF	66%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.4	3.5-5.8cm	AO.R	2.5	3-3.7cm
LVEDD		2-4cm	LA	3.8	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	1.0	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions

normal

LV Wall Thickness

normal

LV Diastolic Function

DD Grade I

SWMA

normal

LA and Ao Root Dimensions

normal

Mitral Valve

normal

Aortic Valve

normal

Tricuspid Valve

normal

Pulmonary Valve

normal

Right Side and Systolic Function

normal

Intracardiac Masses and Effusion

normal

DR.

جستافى

Name: EMAN MOHAMED FEKRI
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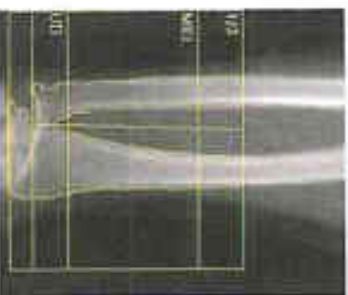
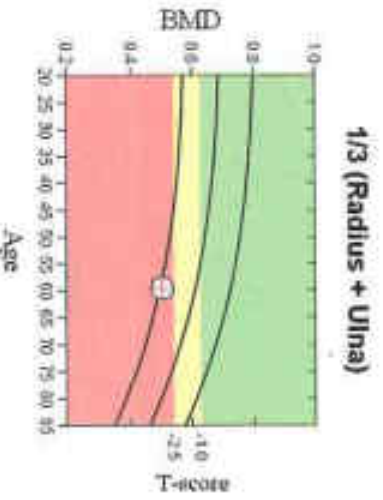


Image not for diagnostic use
228 x 103
DAP: 1.6 cGy*cm²



Radius + Ulna	BMD (g/cm³)	T-score	Z-score
1/3	0.504	-3.1	-1.8

WHO Classification: Osteoporosis
Fracture Risk: High

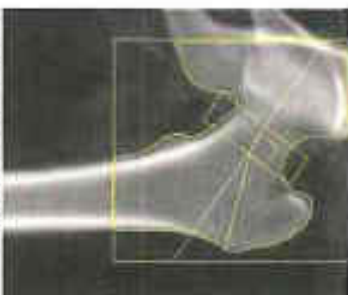
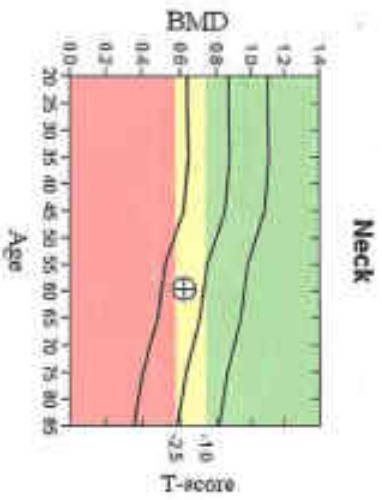


Image not for diagnostic use
97 x 103
NECK: 49 x 15
DAP: 1.7 cGy*cm²



Region	BMD (g/cm³)	T-score	Z-score
Neck	0.620	-2.1	-0.9
Total	0.726	-1.8	-0.8

WHO Classification: Osteopenia

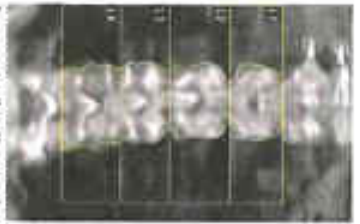
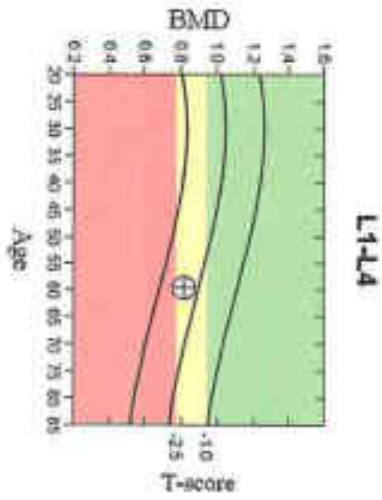


Image not for diagnostic use
116 x 129
DAP: 2.2 cGy*cm²



Region	BMD (g/cm³)	T-score	Z-score
L1-L4	0.816	-2.1	-0.7
L2-L4	0.824	-2.3	-0.9

WHO Classification: Osteopenia
Fracture Risk: Increased

Scan Date: 05 August 2026 - A0805260R

T-score vs. White Female. Source:2012 BMDCS/Hologic
White Female. Z-score vs. Hispanic Female.
Source:NIH/NIES

T-score vs. White Female. Source:2012 BMDCS/Hologic
Z-score vs. Hispanic Female. Source:Hologic

Hologic®

٢١٩٩



الهيئة العامة للتعليمات والمسابقات الصحية
مستشفى أحمد ماهر التخصصي

The General Organization for Teaching Hospitals
and Institutes

Ahmad Maher Teaching Hospital

To whom it may concern

ابن مهندس ري

Mrs. Mr Eman... Mohamed..... years old

female male patient presented with diagnosis

Rt breast... Ca.....

She He was referred to us for (radical / adjuvant / Neoadjuvant /
palliative) Radiotherapy

She He has received (3D / MRT) radiotherapy to
Chest wall

At Dose : 40 Gy / 15 fx / 3 ...w .

Additional (Boost) ; yes/ no at dose Gy / fx / w
(sequential / concomitant)

Started: 5 / 3 / 2026

Ended: 29 / 3 / 2026 With good tolerance .

Head of department

Dr. Hany William

ECHO REPORT

NAME: ليمان محمد فكري DATE: 13/6/2026
ID NUMBER: / SEX: AGE:

EF	62%	55-75%	PWT	0.7	0.6-1.1cm
LVEDD	4.8	3.5-5.8cm	AO.R	2.1	3-3.7cm
LVESD	2.3	2-4cm	LA	3.6	2-4cm
FS	30%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.8	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal
LV Wall Thickness Normal
LV Diastolic Function GI
SWMA No. RWA

LA and Ao Root Dimensions Normal

Mitral Valve N

Aortic Valve N

Tricuspid Valve N

Pulmonary Valve N

Right Side and Systolic Function TAPSE: 2.5

Intracardiac Masses and Effusion

DR. Abdullah

جمهورية مصر العربية
بطاقة تحمل الشخصية

أيمان
محمد فكري محمود محمد
١ ش بروش بكش زهنى
الظاهر - القاهرة

٢٦٦١٢٣١٠٢٠١٩٤٨

KH4369977

٢٠٢٢/١١ ٢٥٥٥٥٥٥٥٠٢٠١٩٤٨

معاملة فرد
أمنى مسلمة

البطاقة سارية حتى ٢٠٢٠/١١/٢٥



نظير بكش زهنى
٢٠٢٠/١١/٢٥

٢٠٢٠/١١/٢٥

٢٠٢٠/١١/٢٥

مبادرة



Presidence Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : **ايمن محمد قنبر محمود**

Sex:

3- Medical number:

Female

83258063

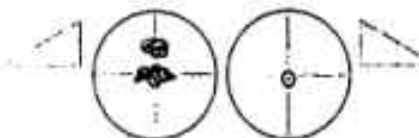
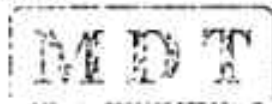
O'd Med :

O'd Med :

326-2025

2-ID number:

Patient Age : 58 Y



Family history

Family history

Phone

Phone

012236249820

BMI

BMI

REASON OF MDT

REASON OF MDT

Marital status

Marital status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

2025-03-08

Co morbidity

Co morbidity

Menopausal status

Menopausal status

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

Mets Workup :

BX
IDC

ER
8/8

PR
4/8

Her 2 Neu
2

SISH

Ki67

Bx COMMENT:

ER COMMENT:

PR COMMENT:

Her 2 Neu COMMENT:

SISH COMMENT:

T
3

N
FNA 0

M
0

Sonomammography

CESM --> RT CENTRAL 12 O'CLOCK MASS 1.8X2 CM

PERI-AREOLAR NON - MASS ENHANCEMENT MICRO CLASSIFICATION + RT INE

PATIENT HISTORY

TM: CEA >>1.14 ... CA15-3>> 7.87... CA125>>6.6 *Benign*

2/2/25 CT CAP >> FEW LT SUB CM SCV LN ...2HFL ... BONE LESIONS(RT ILIAC,L5)

PET/CT>> NO DISTANT METS RT BR CA CT1 CN2 M0 ... MULTICENTRIC INDETERMINE

CONDITION STARTED 9M AGO E RT BR LUMP+ BLEEDING PER NIPPLE

GYN CONSULT>>> FU E NO IMPROVEMENT

INVESTIGATED AT BAHIA >>> RT MULTICENTRIC BR DISEASE TPBC

STILL DIDNOT START ANY TTT COMING FOR PLAN OF MANAGEMENT INCLUDING ANTI HER2

16/2/25 PET/CT >>> METABOLICALLY ACTIVE RT BREAST LESION

NO EVIDENCE OF FDG AVID DISTANT METASTASES

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

FOR NACT TCHP x6C THEN REASSES E SONONMAMO AND PAN CT

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY M. Kellany	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

MDT FORM 1517 / 2025 G

Name	ايما سندرس	MRN	٣٤١ / ٥٥
Age	58 1.0	ID	٥٦٦١٥٢١٠٠٠١٩٨١
Menopausal status		Phone	١١٥٥٣٦ ٥٢٩٨٥
NRS SCORE		Marital status	١٢٩٣٧ ٤٦٤ ٢٥
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX: IDC				R	L
ER: 8/5	PR: 4/8	HER 2: 2	KI67:	TNM	
Sonomammography:					
Dt Finished NACT					
Mets Workup: 6 cycle TC Lst 4-7-2025					
pt received Phesigo 3 cycle Lst 1-7-2025					

Recommendations: Sonography → R

The index mass wasn't clipped now complete Response after index mass + Retro areolar. Residual microcalcifications it was away from the index mass → pt. was initially planned for Rt MRM.

Omnia Essing
13/7/2024

MDT OUTCOME & PLAN

For Rt ~~PHESIGO~~ M. & SMLB.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. محمد النور	Dr. Hossam 5/7/2025				



مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الست المصرية
صحة
مصر

MDT FORM

/

/ 20

G

Name	ايما ن محمد فكري مكي	MRN	157 / C.C
Age	57 yr	ID	
Menopausal status	Natural Post-Menopausal	Phone	
NRS SCORE		Marital status	Divorced
BMI		Family history	Mother → Breast Ca.
CO morbidity	-ve	REASON OF MDT	

* Echo. [23/11/25]: EF: 63%.
* Full lab. OK.
* AUC: 4.3? Clinical Data

BX:

ER:

PR:

HER 2:

KI67:

Sonomammography:

* CESM: Rt. central 12 o'clock Mass 1.8x2cm
peri-areolar non-mass enhancement

Mets Workup: 1* Tumor Markers: - CEA: 9.14
- CA15-3: 7.97
- CA125: 6.6

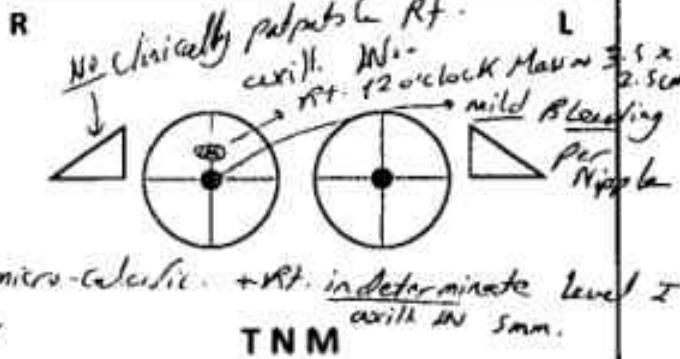
- CT CAP [21/2/25]: - CA15-3: 7.97
- CA125: 6.6

- ? Few Lt. subcentimetric supraclav. LNs.

- ? 2 Hypodense HFs.

- ? Bone lesions (Rt. iliac, L5).

* PET-CT → No Distant Met.



Recommendations:

Rt. Breast Ca. cT₂ cN₁ Ho₊ ER-CT₊ (Triple-Positive) Multicentric in Indeterminate

* Condition started 9 months ago, with Rt. Breast lump + Bleeding per nipple.

* Gynecological consult. → FU with no improvement.

* Investigated at Egypt Viro → Rt. Multicentric Breast disease.

Triple-Positive.

* still Did not start any Tx & coming for Backup management including

MDT OUTCOME & PLAN

anti-HER-2 III
c.c / c / c 2

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name : الأستاذ / ايمان محمد فكرى محمود محمد	Request No. : 13002349
Patient No. : 26612310201948	Request Date : 15/02/2025
Path No. : SR-281/25	Verification Date : 04/03/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 58 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical data:

Breast mass.

Nature of specimen:

Slide and block revision.

Gross examination:

Three paraffin blocks labelled by (8367-24), (505-25) and (1647-24).

Six immunostained slides for (ER, PR, HR2 and three slides for HER2 ISH).

- Four referred films, two of which labeled by (1647-24) and other two labeled (1635-24).

Microscopic examination:

Slides and block labeled by (505-25):

* Histological type: Invasive duct carcinoma.* Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+1) Grade II.

* DCIS: Minimal.

* Angioinvasion: Could not be detected within the limits of biopsy recieved

* IPT (Positive control showed appropriate reactivity):

- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).

- PR: Weak nuclear reaction in <1% of tumor cells (score 2/8).

- Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).

- SISH :Amplified, Her2 copy number 5.2, Her2/CEP 17: 4.2.

Slides and block labeled by (8367)

* Histological type: Invasive duct carcinoma.* Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+1) Grade II.

* DCIS: Focal.

* Angioinvasion: Could not be detected within the limits of biopsy recieved.

* Microcalcifications: Positive.

Verified by :

dr.esraa magdy



إسم المريض ثلاثي:
رقم الملف:



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Follow up / Progress note

Protocol	Day/week Of Protocol	Temperature	WT:	HT:	Blood Pressure	Blood Sugar
			BSA:			

☐ Treatment

☐ Follow-up

C5 pharygo

Clinical findings:

فحص عام: جيد / 17.0 / 7.0 / 100

ملاحظة: 17.0 / 7.0 / 100

ملاحظة: 17.0 / 7.0 / 100

	Doctor Order	الوقت
EF:	pharygo is mandatory next cycle	
BP:	17.0 / 7.0 / 100	
TEMP:	17.0 / 7.0 / 100	
HR:	17.0 / 7.0 / 100	
HB:	17.0 / 7.0 / 100	
WBC:	17.0 / 7.0 / 100	
PLT:	17.0 / 7.0 / 100	
UREA:	17.0 / 7.0 / 100	
CREAT:	17.0 / 7.0 / 100	
ALT:	17.0 / 7.0 / 100	
AST:	17.0 / 7.0 / 100	
T.BIL:	17.0 / 7.0 / 100	
D.BIL:	17.0 / 7.0 / 100	

Doctor Signature: Date: 1 / 20 Time:

اسم المريض ثلاثي:
رقم الملف:
.....



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Oncology Follow up Sheet
صحيفة متابعة مريض أورام (☐ أطفال / ☐ كبار)

الفتة

اسم المريض: اسماء محمد فتحي محمود	
الرقم الطبي: ١٢٢٥٨١٦٣	رقم الملف: ٢٠٢٥ / ٢٤٦
المعاملة المالية:	الرقم القومي: ٤٦٦١٤٢١٠٤٠١٩٤٨
الطبيب المعالج:	
العنوان الدائم: المنطقة الجديدة بشارع زهني الزهاجر القاهرة	
رقم التليفون: ١٩٤٢٦٤٩٨٩	الجنسية: مصرية
النوع: اشري	السن: ٥٨
تاريخ الميلاد: ١٩٦٧ / ١٤ / ٢١	تاريخ اول زيارة: ٢٠٢٥ / ٢ / ١٢
كود الأشعة:	

اسم المريض ثلاثي:
رقم الملف:



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CS
London
+ve FH

post op for Breast c

CT3 + No. Ho by PET-CT
hepate. gdt

FNA
of mass in
liver

III. INVESTIGATIONS:

2 Biopsies

Laboratory Results:

Date: / /20	T BIL: 10CII	Creat:
HB:	D Bill:	Urea: on big
Plt:	AST: En+3/3	ALP:
WBC: Neut:	ALT:	Ca: K:

Date of Echo: / /20

pn+u

LVEF= %

> Tumor Markers:

1 to 2.2t

En+3/3

SIS H: a+4

pn+4/3

1 to 2.2t

C.E.S.T. in 21.1.20

1 to 2.2t

> Other test:

Rt: 120x m 1.8x2w

Appt

Δ Non-men. embolism
+ microcl.

H.O.I. in 3/3/20

J. Net CT TCHP to plain

e+SM ap-CT

> Pathology and immunohistochemistry

7/12/69

HR: 172

اسم المريض ثلاثي:

رقم الملف:



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PERFORMANCE STATUS AT TIME OF PRESENTATION

EF 63.

a/t: RY Refikah nur 4.5.20

TLC 6-5 / 1-75, 44 12-7. flt 304

- **Head and Neck**

for Capt. Taketa, Goshute AUG 5.

4. C + phos Neway

- Chest and Heart

7. Pb T/C 2.9/1.31, Hb 13.1 p/H 20.8

Alt 14.7, AS 9.3, 24.7, 14.7, 0.69

- **Abdomen and Pelvis**

domen and Pelvis

- **Limbs**

تاکوثر (۱۴۰) + ۵-۱۷

CL GSK AUG 5 (47) 6.2.1.1

- Regional Lymph nodes

➤ Site:..... Single: ☐..... Multiple ☐.....

➤ **Fixed:** O..... **Mobile:** O..... **Painless:** O..... **Painful:** O.....

- **Non Regional Lymph nodes**

۱۷۰۰/۱۷۰۰

➤ **Site**

C, HPLC.D

tax of 140m

C, carb. 4600

Neurological findings

Genitalia

..... اسم المريض ثلاثي:
..... رقم الملف:



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Follow up / Progress note

Protocol	Day/week Of Protocol	Temperature	WT:	HT:	Blood Pressure	Blood Sugar
			BSA:			

☐ Treatment

☐ Follow-up

Clinical findings:

Rx bone Care a.s
مريض في السبوع الثاني عشر
Rx ENSURE
2 x 2
في اليومين
ع.ع.ع

Doctor Order		الوقت
EF; %		
BP;		
TEMP;		
HR;		
HB;		
WBC:		
PLT:		
UREA:		
CREAT:		
ALT:		
AST:		
T.BIL:		
D.BIL:		

Doctor Signature: Date: / / 20 Time:

..... اسم المريض ثلاثي:
..... رقم الملف:



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مستشفى اورام دار السلام " هرمل "

Follow up / Progress note

Protocol	Day/week Of Protocol	Temperature	WT:	HT:	Blood Pressure	Blood Sugar
			BSA:			

☒ Treatment

☐ Follow-up

* Full lab: OK.

* Echo. [28/4/2025]: EF: (67%).

Clinical findings:

* For:

(C2) T.C.
Tasterc. 140mg. * L. Carbo. AUC (5)

(C1) Phesgo L 1200mg s.c. Perjeta + 600mg s.i. - Herceptin.

Gastrazole 40mg Tab. once daily for 20 days.

Granitrol 2mg Tab. twice daily for 5 days.

Doctor Order

	الوقت
EF; %	ondaken 2 Bmg patches twice daily for 5 days.
BP;	Methabiozon 8mg Tab. twice daily for 3 days.
TEMP;	CBC → ✓
HR;	FANS → ✓ } (28/4/2025)
HB;	Serum → 1.1
WBC:	CILD Phesgo (1200/600)
PLT:	⊕ C3 TXT/Carbo (AUC=5)
UREA:	CLPh/2 ALG
CREAT:	(28/4/2025)
ALT:	
AST:	
T.BIL:	
D.BIL:	

Doctor Signature: Date: 28/4/2025 Time:

٢٠٢٠/٥/١٩

P دیکھا P کوٹر دلوں P+ / وٹراہ مع س ف م م

EF 671-
2814

C2 HP

+++T / مکتب
C4

4500 AUC S

مکتب
5119

فوجو - تحت الجلد

وہ الفا
بکریہا

جبرائیل
ملہ ۲۲ سابع

عصر
۵/۱۹

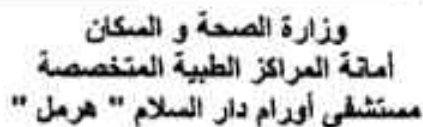
یار آمد

مینا بیو جین

نیرتک

مکتب
+++T Corbo
07/19
نور

مکتب
+++T Corbo
07/19
نور



.....
AUCS



Patient Name : الأستاذة / ايمان محمد فكرى محمود محمد	Request No. : 13002349
Patient No. : 26612310201948	Request Date : 15/02/2025
Path No. : SR-281/25	Verification Date : 04/03/2025
Sex : Female	Financial Category : أورام دار السلام - نفقة النولة
Age : 58 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

* IPT (Positive control showed appropriate reactivity):

- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Moderate nuclear reaction in 5% of tumor cells (score 4/8).
- Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).
- SISH: Amplified.

Films and blocks labeled by (1635, 1647-24): Hemorrhagic smears free of atypical cells.

Diagnosis:

Breast mass, slide and block revision:

Referred material labeled as (505-25)

Invasive duct carcinoma, Grade II .

- ER: Positive (score 8/8).
- PR: Negative (score 2/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified.

Referred material labeled as (8367)

- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified.

Referred films: Hemorrhagic smears free of atypical cells.

Dr. Rana Fathy.

Dr. Duaa Muhammed

Verified by :

dr.esraa magdy



MRN: 10124255885
Patient Name: EMAN MOHAMMED FAKRY MAHMOUD
D.O.B: 31/12/1966
IC No.: 26612310201948
Sex: Female
Location: External Clinic

CYTOPATHOLOGY

(Cytology - 1 Site) فحص خلوي (1 موقع)

Specimen Id	2412260002	Collection Date	26/12/2024 08:18 AM
Accession No	CYT024-1647	Received Date	26/12/2024 08:18 AM
Sample Type	Fluid/ Smear	Doctor	MOHAMMED FAWZEY AHMED MOHAMMED SALEM

Nature of Specimen:

Rt. Axillary Indeterminate lymph node, US guided FNAC.

Gross Picture:

Received 4 unstained smears slides, Pap-stained. One cell block received, H&E-stained.

Microscopic Picture:

Smears and cell block are hypocellular show reactive lymphocytes at different stages of maturation with scattered tangible body macrophages. No atypical or malignant cells detected.

Diagnosis:

Rt. Axillary lymph node, US guided FNAC:

Hypocellular, consistent with reactive lymphoid cells. Negative for malignancy.

Comments:

Recommended for clinico-radiological correlation.

Category Code (LH)

C2: Benign: benign lymphoid cells

Signature

Mona Alabrak, MD.

PATIENT NAME: EMAN MOHAMMED FAKRY MAHMOUD

GENDER: Female

DATE: 21-Jan-2025 06:30 PM

PATIENT ID: 10124255695

AGE: 58 y

ORDER: 3217757

CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY**INDICATION:**

- Rt breast pathologically proven IDC

TECHNIQUE:

- Contrast Enhanced Spectral Mammography CC and MLO views

REPORT:

- Contrast Enhanced Spectral Mammography showed mild symmetrical back ground parenchymal enhancement.
- Rt central outer 12 o'clock paraareolar mass showing moderate heterogenous mass enhancement; measured 1.8 x 2 cm with clumped non mass enhancement seen extending from the mass anteriorly to nipple periareolar region with overlying grouped microcalcification
- No lt breast mass or non mass enhancement.
- Normal skin thickness and contour.
- Normal Lt nipple areola complexes.

OPINION:

- Rt breast mass with suspicious enhancement pathologically proven IDC (BIRADS 6).
- No lt breast pathological mass or non mass enhancement (BIRADS 2)

Reported By,

Dr. Radwa Magdy, MSc

21-Jan-2025 07:10 PM



PATIENT NAME: EMAN MOHAMMED FAKRY MAHMOUD

GENDER: Female

DATE : 02-Feb-2025 02:58 PM

PATIENT ID: 10124255695

AGE: 58 y

ORDER: 3234395,3234430

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS WITH TRIPHASIC STUDY OF THE LIVER**

FINDINGS:

1- CHEST:

- Heterogenous nodular enhancement of the right breast parenchyma.
- Few bilateral subcentimetric axillary level I lymph nodes, the largest on the right side measuring about 12 X 10 mm.
- Few subcentimetric left supraclavicular lymph nodes, the largest on measuring about 4 mm.
- Few bilateral tiny millimetric pulmonary nodules most are calcified.
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS:

- Mildly enlarged liver harboring segment VI & VIII hypodense non enhancing focal lesions with CT attenuation value of about 10 HU. No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Calculus gall bladder with large radiodense stone measuring about 13 mm.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- Normal appearance of the pancreas, spleen, kidneys and adrenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- No abdominal or pelvic masses or collections.

PATIENT: EMAN MOHAMMED FAKRY MAHMOUD, 10124255695, Female, 58 y

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10602

مركز هيئة الشرح
شركة تشخيصية
مستشفى سويدي الشريعة
الطبيعي - القاهرة - مصر

- No sizable solid or cystic adnexal mass lesions.
- Prominent left ovarian vein with dilated tortuous left parametrial vessels associated narrow aorto-mesenteric distance measuring about 5 mm.

The revised bone window revealed:

1. No CT evidence of vertebral collapse or extra-osseous soft tissue component.
2. Right iliac bone and right L5 transverse process sclerotic foci.
3. Right hemisacralized L5 vertebra.
4. Bilateral sacroiliitis.

OPINION:

- *Heterogenous nodular enhancing right breast neoplastic parenchyma.*
- *Enlarged liver with segment VI & VIII hypodense non enhancing focal lesions, likely of benign nature "cyst".*
- *No suspicious pulmonary nodules.*

Reported By,

Dr. Sara Youary, MSc

Dr. Salwa Yahya, MD

02-Feb-2025 07:17 PM



Echo report

Name: اسمان محمد فكري

Date: 23/2/ 2025

Age:

٢٥
٢٥

EF	<u>63%</u>	54-74%	PWT	<u>9</u>	0.6-1.1cm
LVEDD	<u>٤٩</u>	3.5-5.7cm	AO	<u>33</u>	2.0-3.7cm
LVESD	<u>33</u>	2.2-4.0cm	LA	<u>36</u>	1.9-4.0cm
FS	<u>33</u>	25-45%	RV		0.7-2.3cm (supine)
SWT	<u>8</u>	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions:

LV wall thickness:

LV diastolic function:

SWMA.

LA size

Aortic root dimension

Mitral Valve

Aortic valve

Tricuspid valve

Pulmonary valve

Right side size and systolic function

Intracardiac masses and thrombi

Pericardium

IVC diameter and collapsibility.

Conclusion:

D/Mostafa Shahien

23/2/25

63%
DDCET

PATIENT NAME: EMAN MOHAMMED FAKRY MAHMOUD
GENDER: Female
DATE : 25-Dec-2024 02:29 PM

PATIENT ID: 10124255695
AGE: 57 y
ORDER: 3178750

ULTRASOUND GUIDED FNAC

Clinical history

- Indeterminate right axillary lymph node.

BIOPSY

- U/S guided FNA (slides and cell block) was done from right indeterminate axillary lymph node, for cytological assessment.

No Complications

Procedure well tolerated

Reported By,

dr. HANAN ALI ELDIN, MSc

25-Dec-2024 02:57 PM



PATIENT NAME: EMAN MOHAMMED FAKRY MAHMOUD

GENDER: Female

DATE : 21-Jan-2025 06:30 PM

PATIENT ID: 10124255695

AGE: 58 y

ORDER: 3217757

CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY

INDICATION:

- Rt breast pathologically proven IDC

TECHNIQUE:

- Contrast Enhanced Spectral Mammography CC and MLO views

REPORT:

- Contrast Enhanced Spectral Mammography showed mild symmetrical back ground parenchymal enhancement.
- Rt central outer 12 o'clock paraareolar mass showing moderate heterogenous mass enhancement measured 1.8 x 2 cm with clumped non mass enhancement seen extending from the mass anteriorly to nipple perareolar region with overlying grouped microcalcification
- No lt breast mass or non mass enhancement.
- Normal skin thickness and contour.
- Normal Lt nipple areola complexes.

OPINION:

-R: breast mass with suspicious enhancement pathologically proven IDC (BIRADS 6).

- No lt breast pathological mass or non mass enhancement (BIRADS 2)

Reported By,

Dr. Radwa Magdy, MSc

21-Jan-2025 07:10 PM



PATIENT NAME: EMAN MOHAMMED FAKRY MAHMOUD

GENDER: Female

DATE : 02-Feb-2025 02:56 PM

PATIENT ID: 10124255695

AGE: 58 y

ORDER: 3234395,3234450

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS WITH TRIPHASIC STUDY OF THE LIVER****FINDINGS:****1- CHEST:**

- Heterogenous nodular enhancement of the right breast parenchyma.
- Few bilateral subcentimetric axillary level I lymph nodes, the largest on the right side measuring about 12 X 10 mm.
- Few subcentimetric left supraclavicular lymph nodes, the largest on measuring about 4 mm.
- Few bilateral tiny millimetric pulmonary nodules most are calcified.
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS:

- Mildly enlarged liver harboring segment VI & VIII two hypodense non enhancing focal lesions with CT attenuation value of about 10 HU. No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Calculus gall bladder with large radiodense stone measuring about 13 mm.
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- No pathologically enlarged abdominal or pelvic lymph nodes.
- No abdominal or pelvic masses or collections.

PATIENT: EMAN MOHAMMED FAKRY MAHMOUD, 10124255695, Female, 58 y

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OPINION:

- *Heterogenous nodular enhancing right breast neoplastic parenchyma.*
- *Enlarged liver with segment VI & VII hypodense non enhancing focal lesions, likely of benign nature "cyst".*
- *No suspicious pulmonary nodules.*

Reported By,

Dr. Sara Yousry, MSc

Dr. Salwa Yahya, MD

02-Feb-2025 07:17 PM



PATIENT NAME: EMAN MOHAMMED FAKRY MAHMOUD
GENDER: Female
DATE : 21-Jan-2025 06:45 PM

PATIENT ID: 10124255895
AGE: 58 y
ORDER: 3217758

ULTRASOUND GUIDED BIOPSY

BIOPSY

Under aseptic conditions 5 core biopsies from Rt breast pathologically proven malignant lesion (BIRADS 6) for HER2.

No Complications
Procedure well tolerated

Reported By,

Dr. Radwa Magdy, MSc 21-Jan-2025 07:51 PM



PATIENT NAME: EMAN MOHAMMED FAKRY MAHMOUD
GENDER: Female
DATE : 25-Dec-2024 01:48 PM

PATIENT ID: 10124255895
AGE: 57 y
ORDER: 3178751

Ultrasonnd Guided Biopsy

BIOPSY

Under aseptic conditions multiple core biopsies from right breast highly suspicious lesion (BIRADS 4) for histopathological confirmation, ER, PR and HER2.

No Complications
Procedure well tolerated

Reported By,

dr. HANAN ALI ELDIN, MSc 25-Dec-2024 02:55 PM

